

# **SALIC OINTMENT**

## **0.05% Betamethasone, 3% Salicylic Acid**

### **Product Description**

A white and thick ointment pack in 15g boxed tube.

### **Composition**

Each gram of Salic Ointment contains 30mg of Salicylic Acid and 0.64mg of Betamethasone Dipropionate (0.5mg of Betamethasone).

### **Mode of Action**

Betamethasone dipropionate is a synthetic fluorinated corticosteroid. It is active topically and produces a rapid and sustained response in inflammatory dermatoses. It is also effective in less responsive conditions e.g. scalp psoriasis, chronic plaque psoriasis of the hands and feet. Salicylic acid has keratolytic action. It softens keratin, loosens cornified epithelium and desquamates the epidermis. It aids in the penetration of betamethasone.

### **Pharmacodynamics and pharmacokinetics**

#### **Pharmacodynamic**

Salic preparations contain the dipropionate ester of betamethasone which is a glucocorticoid exhibiting the general properties of corticosteroids, and salicylic acid which has keratolytic properties.

Salicylic acid is applied topically in the treatment of hyperkeratotic and scaling conditions where its keratolytic action facilitates penetration of the corticosteroid.

In pharmacological doses, corticosteroids are used primarily for their anti-inflammatory and/or immune suppressive effects.

Topical corticosteroids such as betamethasone dipropionate are effective in the treatment of a range of dermatoses because of their anti-inflammatory, anti-pruritic and vasoconstrictive actions. However, while the physiologic, pharmacologic and clinical effects of the corticosteroids are well known, the exact mechanisms of their action in each disease are uncertain.

#### **Pharmacokinetics**

Salicylic acid exerts only local action after topical application.

The extent of percutaneous absorption of topical corticosteroids is determined by many factors including vehicle, integrity of the epidermal barrier and the use of occlusive dressings.

Topical corticosteroids can be absorbed through intact, normal skin. Inflammation and/or other disease processes in the skin may increase percutaneous absorption.

Occlusive dressings substantially increase the percutaneous absorption of topical corticosteroids.

Once absorbed through the skin, topical corticosteroids enter pharmacokinetic pathways similar to systemically administered corticosteroids. Corticosteroids are bound to plasma proteins in varying degrees, are metabolised primarily in the liver and excreted by the kidneys. Some of the topical corticosteroids and their metabolites are also excreted in the bile.

### **Indications**

Salic Ointment is indicated for the relief of inflammations of hyperkeratotic and other skin conditions such as: chronic atopic dermatitis, psoriasis, eczema (including nummular eczema, eczematous dermatitis, hand eczema), lichen planus, neurodermatitis, seborrheic dermatitis of the scalp, ichthyosis vulgaris (including other ichthyosis conditions) and dyshidrosis.

### **Recommended Dosage and directions for use**

Apply a thin film twice daily; in the morning and at night. Less frequent application is adequate for maintenance in some patients or as prescribed by the physician.

**Mode of Administration**

Salic Ointment is administered by topically

**Contraindications**

Patient with a history of sensitivity reactions to any of the components of Salic Ointment.

**Warnings and Precautions**

Treatment should be discontinued if irritation or sensitization occurs.

In presence of an infection, appropriate therapy is indicated.

Any of the adverse reactions that are reported on the systemic use of corticosteroid including adrenal suppression, may also occur with topical corticosteroids, especially in infants and children.

Systemic absorption of topical corticosteroids or salicylic acid will be increased if extensive body surface areas are treated or if the occlusive technique is used.

Discontinue the use of this preparation if excessive dryness or increased skin irritation develops.

Avoid contact with eyes and mucous membranes.

**Pediatric patients** may demonstrate greater susceptibility to topical corticosteroid-induced hypothalamic-pituitary-adrenal (HPA) axis suppression and to exogenous corticosteroid effects than mature patients because of greater absorption due to a large skin surface area to body weight ratio.

HPA axis suppression, Cushing's syndrome, linear growth retardation, delayed weight gain and intracranial hypertension have been reported in children receiving topical corticosteroids. Manifestations of adrenal suppression in children include low plasma cortisol levels and absence of response to ACTH stimulation. Manifestations of intracranial hypertension include a bulging fontanelle, headaches and bilateral papilledema

**Interaction with Other medicaments**

There are no known drug interactions.

**Statement on usage during pregnancy and lactation**

Safety of topical corticosteroids use in pregnant women has not been established, drugs of this class should be used during pregnancy only if the benefit outweighs the risk to the fetus. It should not be used extensively in large amounts or for prolonged periods of time in pregnant patients.

It is not known whether topical application of corticosteroids can result systemic absorption to be present in breast milk.

**Adverse Effects/ Undesirable Effects**

Salic Ointment preparations are generally well tolerated and side-effects are rare.

Adverse reactions that have been reported with the use of topical corticosteroids include itching, burning, dryness, irritation, acneiform eruptions, folliculitis, hypertrichosis, hypopigmentation, allergic contact dermatitis and perioral dermatitis.

Maceration of the skin, secondary infection, skin atrophy, miliaria and striae may occur more frequently with the use of occlusive dressing.

In addition, prolonged use of salicylic acid may cause dermatitis.

**Overdose and Treatment**

Excessive or prolonged use of topical corticosteroids can cause adrenal insufficiency and may manifest hypercorticism including Cushing's disease.

Excessive or prolonged use of salicylic acid may cause symptoms of salicylism.

**Storage conditions**

Protect from light and store below 30°C. Keep out of reach of children

**Dosage Forms and packaging available**

This ointment is available in 15g tube

**Shelf-life**

The Ointment can be used within 60 months from the date of manufacture if kept as recommended.

**Registration number**

**SALIC OINTMENT – MAL12025053AZ**

**Name and Address of Manufacturer**

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**Date of revision of package insert**

**May 2016**